

7.35.3 NMAC: proposed amendments to the practicum and training provisions

Working draft v7, July 26, 2026. Rule hearing August 28, 2026.

- This document analyzes the recommendation of Dr. Anne Metz to the Training and Education Committee dated July 17, 2026, and states it as amendment language against the proposed rule published July 23, 2026, for the committee's consideration.
- It covers 7.35.3.19, practicum requirements, and the three provisions on which the practicum depends: 7.35.3.18, educational requirements; 7.35.3.14, authorized possession; and Paragraph (5) of Subsection H of 7.35.3.20, staffing ratios. One new section is proposed; no other provision of Part 3 is addressed.
- The working redline of Denali Wilson dated July 25, 2026 is folded in. Provisions taken from it are cited to her. Where she and the Metz recommendation diverge, both are shown and the question is stated.
- Recommendation 1 of the Metz recommendation, retitling "practitioner" as "licensed provider", is carried through. The retitle is marked wherever it falls inside a provision reproduced here. Addendum C maps every other place in 7.35.3 NMAC and 7.35.2 NMAC that a conforming amendment would have to reach.
- The Metz recommendation states several requirements as ranges. This draft adopts the low end of each range and marks it, so that the committee may raise it.
- Each proposed change carries a citation to its source. Where the rule as published is unclear, the published text is left as it stands and the question is stated at that provision.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged.

Metz: 6 to 8

Hours Published against proposed

COMPONENT	PUBLISHED	PROPOSED	CHANGE
New Mexico module	not stated	6	+6
Psilocybin therapy module, didactic	30	68	+38
Simulated patient experience	5	5	0
Role-specific module	5	5	0
Module total, either permit	40	84	+44
Practicum, facilitator	100	62	-38
Practicum, practitioner	120	72	-48
Supervision or consultation	10	20	+10
Program total, facilitator	150	166	+16
Program total, practitioner	170	176	+6

The practitioner rows take the 20 supervision hours in 7.35.3.19 C to be within the published 120. See 7.35.3.19 C.

7.35.3.14 Authorized possession, purchase, or sale of medical psilocybin

Authorized possession. Amended so that a student may lawfully be provided psilocybin in the first stage of the practicum, and to conform the permit title.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in

7.34.3.14 AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY PRACTITIONERS, FACILITATORS, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a practitioner, facilitator, or healing center shall enable practitioners, facilitators, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

Section heading and lead-in

~~7.34.3.14~~ **7.35.3.14** AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY ~~PRACTITIONERS~~ **LICENSED PROVIDERS**, FACILITATORS, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a ~~practitioner~~ **licensed provider**, facilitator, or healing center shall enable ~~practitioners licensed providers~~, facilitators, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

SOURCE Two changes. Correction: the heading as published reads 7.34.3.14; the history note at the end of the same section reads 7.35.3.14. Permit title: Metz recommendation, page 1, recommendation 1. Addendum C.

Subsection A, Practitioners

(A) Practitioners: Practitioners may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients during administration sessions conducted at a healing center location or other approved location. A practitioner may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

Subsection A, Practitioners

(A) ~~Practitioners~~ **Licensed providers**: ~~Practitioners~~ **Licensed providers** may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, during administration sessions conducted at a healing center location or other approved location. A ~~practitioner~~ **licensed provider** may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

SOURCE Consequential on proposed 7.35.3.29. The permit title is also conformed.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients in administration sessions conducted at healing centers and other approved locations.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, in administration sessions conducted at healing centers and other approved locations.

SOURCE Consequential on proposed 7.35.3.29.

Subsection C, Healing centers

(C) Healing centers: Owners and employees of healing centers who are registered with the department may purchase medical psilocybin products from permitted producers, may possess medical psilocybin products, and may sell or otherwise administer those products to qualified patients in administration sessions conducted at the healing center or other approved locations, provided that such individuals are also designated to engage in each activity by the healing center. A healing center may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

Subsection C, Healing centers

Not amended.

SOURCE Not amended.

Please review. This subsection conditions healing center staff authority on being registered with the department. No such registration appears in 7.35.3 NMAC or in 7.35.2 NMAC.

7.35.3.18 Educational requirements for certifying clinicians, practitioners, and facilitators

Educational requirements. The 84-hour total is stated, the New Mexico module is given an hour count, content areas are added to the required topic list, and the permit title is conformed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.35.3.18 EDUCATIONAL REQUIREMENTS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

Section heading

7.35.3.18 EDUCATIONAL REQUIREMENTS FOR CERTIFYING CLINICIANS, ~~PRACTITIONERS~~ **LICENSED PROVIDERS**, AND FACILITATORS:

SOURCE Permit title only. Metz recommendation, page 1, recommendation 1: retitle "Practitioner" as "Licensed Provider". Addendum C.

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, practitioner, and facilitator certification: All certifying clinicians, practitioners, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department prior to applying for certification, which shall include at a minimum:

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, ~~practitioner~~ **licensed provider**, and facilitator certification: All certifying clinicians, ~~practitioners~~ **licensed providers**, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department, **consisting of a minimum of six didactic hours**, Metz: 6 to 8 prior to applying for certification, which shall include at a minimum: *Paragraphs (1) through (5) not amended.*

SOURCE Metz recommendation, page 2, curriculum table: New Mexico module, 6 to 8 hours.

Please review. The rule sets no date by which the department must create or publish this module. Every certification pathway is conditioned on it.

Subsection B, certifying clinician module

Not amended.

Subsection B, certifying clinician module

Not amended.

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial practitioner and facilitator certification: All practitioners and facilitators shall complete a psilocybin therapy module consisting of a minimum of 30 didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial ~~practitioner~~ licensed provider and facilitator certification: All ~~practitioners~~ licensed providers and facilitators shall complete a psilocybin therapy module consisting of a minimum of ~~30~~ 68 didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

SOURCE Arithmetic. Metz recommendation, page 1, states a total of 84 hours. 84 less Subsection A (6), less Subsection D or E (5, unchanged), less the simulated patient experience (5, unchanged) leaves 68.

Please review. 68 follows from the 84-hour total after Subsection A, Subsection D or E, and the simulated patient experience. The Wilson redline reads the same 84 differently and sets this subsection at 80 didactic hours, striking the simulated patient experience from the lead-in. Which reading of the 84 governs?

Subsection C, required topics

- (1) Overview of practitioner/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) A simulated patient experience of no less than 5 hours; and
- (17) Evaluation to demonstrate competency in the above areas.

Subsection C, required topics

- (1) Overview of ~~practitioner~~ licensed provider/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care;
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) End-of-life and palliative care;
- (17) Screening, suicidality, and crisis response;
- (18) Substance use disorder;
- (19) Post-traumatic stress disorder;
- (20) Somatic awareness and therapeutic touch;
- (21) History and traditional use of psilocybin and other psychedelics;
- (22) Individual and group treatment models;
- (23) Challenging experiences, including hallucinogen persisting perception disorder;
- ~~(16)~~ (24) A simulated patient experience of no less than 5 hours; and
- ~~(17)~~ (25) Evaluation to demonstrate competency in the above areas.

SOURCE Metz recommendation, pages 2 and 3, curriculum table, for paragraphs (16), (17), (20), (21), (22) and (23). July 17, 2026 Training and Education Committee transcript for paragraphs (18) and (19); both are qualifying conditions under Section 3(l) of the Medical Psilocybin Act. The Wilson redline adds the same content areas.

Please review. The Metz recommendation gives an illustrative range for each content area and states that the binding minimum is the total, so no per-topic minimum is drafted. The Wilson redline reaches the same conclusion: “I haven’t added the specific hour requirements for each because I think that is a bit much for what needs to live in regulation” (comment 13). Ranges: core psychotherapy skills and ethics 25 to 30; end-of-life and palliative care 10 to 15; trauma 8; medicine, dosing and clinical research literacy 8 to 10; New Mexico module 6 to 8; treatment models 6 to 8; screening, suicidality and crisis response 5 to 8; somatic awareness and therapeutic touch 4 to 6; history and traditional use 3 to 4; challenging experiences 2 to 4. Should any be drafted as a minimum? No hour figure is given for post-traumatic stress disorder or substance use disorder.

Subsection D, additional facilitator module <i>Not amended.</i>	Subsection D, additional facilitator module <i>Not amended.</i>
Subsection E, additional Practitioner module, lead-in E. Additional requirements for initial practitioner certification: All practitioners shall complete a module on psychedelic and psilocybin therapeutic approaches consisting of a minimum of five didactic hours, prior to applying for certification, which shall include:	Subsection E, additional Practitioner module, lead-in E. Additional requirements for initial practitioner <u>licensed provider</u> certification: All practitioners <u>licensed providers</u> shall complete a module on psychedelic and psilocybin therapeutic approaches consisting of a minimum of five didactic hours, prior to applying for certification, which shall include: <i>Paragraphs (1) through (3) not amended.</i> ----- SOURCE Permit title only. Metz recommendation, page 1, recommendation 1. Addendum C. The five didactic hours are not amended.
Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner and facilitator trainings: All practitioners and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in:	Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner <u>Licensed provider</u> and facilitator trainings: All practitioners <u>licensed providers</u> and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in: <u>complete and maintain the following prior to applying for medical psilocybin certification:</u> <i>Paragraphs (1) and (2) not amended. Corrects a sentence that does not complete as published. No obligation changes.</i> ----- SOURCE Correction. The sentence as published does not complete. The permit title is also conformed.
Subsection G, continuing education G. Continuing education requirements for certifying clinicians, practitioners and facilitators: (1) Certifying clinicians: Certifying clinicians shall complete a minimum of eight hours of continuing medical education credits specific to psychedelic medicine or therapy every two years. (2) Practitioners and facilitators: Practitioners and facilitators shall complete a minimum of 20 hours of continuing education credits specific to psychedelic therapy and practice every two years. Practitioners and facilitators shall also keep current with their basic life support, or cardiopulmonary resuscitation and automated external defibrillation certification, or emergency medical technician licensure in addition to the 20 hours of continuing education credits.	Subsection G, continuing education G. Continuing education requirements for certifying clinicians, practitioners <u>licensed providers</u> and facilitators: (1) Certifying clinicians: Certifying clinicians shall complete a minimum of eight hours of continuing medical education credits specific to psychedelic medicine or therapy every two years. (2) Practitioners <u>Licensed providers</u> and facilitators: Practitioners <u>Licensed providers</u> and facilitators shall complete a minimum of 20 hours of continuing education credits specific to psychedelic therapy and practice every two years. Practitioners <u>Licensed providers</u> and facilitators shall also keep current with their basic life support, or cardiopulmonary resuscitation and automated external defibrillation certification, or emergency medical technician licensure in addition to the 20 hours of continuing education credits. ----- SOURCE Permit title only. Metz recommendation, page 1, recommendation 1. Addendum C. The 20 continuing education hours and the life support requirement are not amended.

Subsection H, total module hours

There is no current language. This provision does not exist in the proposed rule as published.

Subsection H, total module hours

H. Total module hours: The requirements of Subsections A, C and D of this section shall together total a minimum of 84 hours for a facilitator applicant. The requirements of Subsections A, C and E of this section shall together total a minimum of 84 hours for a licensed provider applicant. The hours stated for each module in this section are minimums within that total.

SOURCE Metz recommendation, page 3: "Component ranges are illustrative; the binding minimum is the total."

7.35.3.19 Practicum requirements for practitioners and facilitators

Practicum requirements. The practicum totals, the supervision hours and the practicum sequence are amended; independent medical screening, the consultation requirement and the end-of-life checkpoint are added; the section is re-lettered A through K.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.35.3.19 PRACTICUM REQUIREMENTS FOR PRACTITIONERS AND FACILITATORS:

Section heading

7.35.3.19 PRACTICUM REQUIREMENTS FOR ~~PRACTITIONERS~~ LICENSED PROVIDERS AND FACILITATORS:

SOURCE Permit title only. Metz recommendation, page 1, recommendation 1. Addendum C.

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a practitioner or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of 100 hours of supervised practice training for facilitators and 120 hours for practitioners and shall be completed prior to applying for certification. Students shall participate in a minimum of 80 hours of administration day sessions, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria:

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a ~~practitioner~~ licensed provider or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of ~~100~~ 62 ^{Metz: approx. 62} hours of supervised practice training for facilitators and ~~120~~ 72 ^{Metz: approx. 72} hours for ~~practitioners~~ licensed providers and shall be completed prior to applying for certification. Students shall participate in ~~a minimum of 80 hours of administration day sessions, where students are~~ administration day sessions as required by Subsection D of this section, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria: *Paragraphs (1) and (2), the patient and session minimums, not amended.*

SOURCE Metz recommendation, page 4: “approximately 62 hours for Facilitators (steps 1-3); approximately 72 hours for Licensed Providers (steps 1-4).” The permit title is also conformed.

Please review. Three points. The published phrase “half of the didactic requirements” does not identify half of what; the Wilson redline strikes the entry gate entirely, this draft leaves it as published. The published figure of 80 hours of administration day sessions cannot sit within a 62-hour practicum, so it is struck and the step hours in Subsection D govern. The published minimums of 14 different patients over eight different sessions are carried forward here; the Wilson redline strikes them. Do they survive a 62-hour practicum?

Subsection B, minimum practicum hours; preparation and integration sessions B. Minimum practicum hours; in-person and integration sessions: Students shall participate in a minimum of 20 hours of in-person preparation and integration sessions, where students are provided the opportunity to experience, observe, or conduct (when licensure allows) preparation or integration sessions. This shall include a minimum of six different patients during individual sessions. This shall include at least one group preparatory and one group integration session with a minimum of four or more patients.	Subsection B, minimum practicum hours; preparation and integration sessions <i>Not amended.</i> ----- SOURCE Not amended.
Subsection C, Practitioner supervision hours C. Practitioner supervision hours: Practitioners shall complete an additional minimum of 20 hours as a practitioner supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group.	Subsection C, Practitioner supervision hours C. Practitioner <u>Licensed provider</u> supervision hours: Practitioners <u>Licensed providers</u> shall complete an additional minimum of 20 <u>10</u> Metz: 10 hours as a practitioner <u>licensed provider</u> supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group. ----- SOURCE Metz recommendation, page 4, step 4: “Supervisory hours, Licensed Providers only (10 hours).” Please review. Subsection A sets 120 hours for a practitioner. This subsection adds “an additional minimum of 20 hours” without identifying what it is additional to. Within the 120, the published practitioner total is 170. On top of the 120, it is 190. The Metz totals of approximately 62 and 72 differ by exactly her 10 supervisory hours, so the recommendation treats them as within. This draft follows the recommendation, giving a proposed total of 176. Which reading governs?

Subsection D, practicum sequence

There is no current language. This provision does not exist in the proposed rule as published.

Subsection D, practicum sequence

D. Practicum sequence: The practicum required by Subsection A of this section consists of the following, in sequence:

(1) Initial facilitation experience: students shall begin the practicum by completing a minimum of two sessions as a facilitator in a retreat or peer-support model with well participants, in accordance with 7.35.3.29 NMAC. Practicum session experience shall include preparation, administration, and integration; Metz: approx. 30 hours

(2) Co-facilitation experience: after completing the initial facilitation experience, students shall complete a minimum of two sessions co-facilitated with a department-permitted licensed provider, with patients presenting a low-acuity qualifying condition. Practicum session experience shall include preparation, administration, and integration;

Metz: approx. 20 hours

(3) Group practicum: direct participation in group preparation, administration, and integration sessions; and Metz: approx. 12 hours

(4) Additional experience: students may complete the remaining balance of the hours required by Subsection A of this section in any manner of co-facilitation designed by the educational program or healing center hosting the practicum that otherwise complies with this rule. For licensed provider applicants, the supervision hours required by Subsection C of this section count toward that balance.

SOURCE Metz recommendation, pages 3 and 4, steps 1 through 4, for the sequence and the hour figures. Wilson redline for the drafting: the steps are stated as sessions, and Paragraph (4) carries the remaining balance of hours, to be completed "in any manner of co-facilitation designed by the school or healing center hosting practicums that otherwise complies with the regulations of these provisions" (Wilson redline).

Please review. The Metz recommendation states each step in hours and the Wilson redline states the first two in sessions. Both are carried: the session minimums are drafted, the Metz hour figures are shown alongside, and Paragraph (4) carries the balance. The Wilson redline sets the totals at 60 and 70; the Metz figures of 62 and 72 are the ones drafted at Subsection A.

Subsection E, practicum location (published Subsection D)

D. Practicum location: All practicum hours shall take place in an approved healing center or other approved location.

Subsection E, practicum location (published Subsection D)

~~D.~~ **E. Practicum location: All practicum hours shall take place in an approved healing center or other approved location. An approved supervisor at an approved location may host practicum students who are not enrolled at a co-located training program.**

SOURCE Metz recommendation, page 4: "an approved supervisor at an approved location can host practicum students who are not enrolled at a co-located training program."

Subsection F, independent medical screening <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection F, independent medical screening <u>F. Independent medical screening required: A training program overseeing practicum treatments shall not also medically screen patients. Medical screening shall be conducted independently by the clinician with the hosting healing center or another certifying clinician not affiliated with the training program.</u> SOURCE Wilson redline, new subsection: "A training program overseeing practicum treatments cannot also medically screen patients." Please review. From the Wilson redline. The stated concern is that a training program has an inherent interest in its own students being medically cleared so that practicum hours can be completed.
Subsection G, practicum standards (published Subsection E) E. Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice.	Subsection G, practicum standards (published Subsection E) E. <u>G.</u> Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice. <i>Re-lettered only. Text not amended.</i> SOURCE Not amended. Re-lettered.
Subsection H, supervision and consultation; case presentation sign-off <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection H, supervision and consultation; case presentation sign-off <u>H. Supervision and consultation; case presentation sign-off:</u> <u>(1) An applicant for certification shall complete a minimum of 20 hours of supervision or consultation after the practicum is completed, in addition to the practicum hours required by this section;</u> Metz: 20 to 30 <u>(2) sign-off is tied to case presentation. The applicant shall present a minimum of two cases of clients the applicant has personally worked with using regulated medicine. Each case presentation shall take the form of a biopsychosocial case conceptualization, including a discussion of presenting concerns, risk factors, supportive factors, treatment considerations, and recommendations for aftercare; and</u> <u>(3) the consultant or supervisor shall complete a standardized evaluation form on each case presented.</u> SOURCE Metz recommendation, pages 4 and 5: 20 to 30 hours of supervision or consultation, sign-off tied to two presented cases with a standardized evaluation form. Please review. The Wilson redline instead replaces the 10 mentoring hours at 7.35.3.17 A with two student case consultations, stating the requirement in cases rather than hours and striking the 12-month validity period. For Dr. Metz, Ms. Wilson and Dr. Leeman: hours, cases, or both, and does the requirement sit here or at 7.35.3.17 A?

Subsection I, end-of-life practice <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection I, end-of-life practice <u>I. End-of-life practice: Before serving end-of-life participants independently, a licensed provider or facilitator shall complete at least one co-facilitated end-of-life case and present at least one end-of-life case in supervision or consultation.</u> SOURCE Metz recommendation, page 5, end-of-life checkpoint.
Subsection J, waiver of practicum requirements (published Subsection F) F. Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program.	Subsection J, waiver of practicum requirements (published Subsection F) F. J. Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program. <i>Re-lettered only. Text not amended.</i> SOURCE Not amended. Re-lettered.
Subsection K, waiver for applications received by December 31, 2027 (published Subsection G) G. Waiver of practicum hours requirement for applications received by December 31, 2027: An applicant for certification shall not be required to satisfy the full New Mexico practicum hours requirement if the applicant: (1) Applies for certification by December 31, 2027; (2) Completes the didactic requirements by December 31, 2027; (3) Graduates from an educational program that the department certifies by December 31, 2027 or that the department has included on the department-approved list of educational programs by December 31, 2027; and (4) Demonstrates completion of at least 40 hours of contact time through logs or other records of the sessions, including: (a) A minimum of two separate individual sessions including the appointments for preparation, administration and integration; and (b) A minimum of one group session including the appointments for preparation, administration, and integration.	Subsection K, waiver for applications received by December 31, 2027 (published Subsection G) G. K. Waiver of practicum hours requirement for applications received by December 31, 2027: <i>Re-lettered only. Paragraphs (1) through (4) not amended.</i> SOURCE Not amended. Re-lettered. Please review. Paragraph (4)(b) of this subsection requires “A minimum of one group session” while Subsection D(1) of 7.35.3.10 requires “two separate group sessions” for the same 40 hours of contact time.

7.35.3.20 Requirements for healing centers and other approved locations

Requirements for healing centers and other approved locations. The section heading number is corrected and the permit title is conformed in Paragraph (5) of Subsection H. No staffing ratio is changed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in

7.34.3.20 REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements:

Section heading and lead-in

~~7.34.3.20~~ **7.35.3.20** REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements: *Subsections A through G and I not amended. Of Subsection H, only Paragraph (5) is reproduced.*

SOURCE Correction. The heading as published reads 7.34.3.20; the history note at the end of the same section reads 7.35.3.20.

Subsection H(5), staffing ratios

(5) Includes procedures to ensure that there are practitioners and facilitators present at all times during an administration session with a minimum of one practitioner and one facilitator for individual patient sessions; and in group administration sessions a minimum of one practitioner for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated.

Subsection H(5), staffing ratios

(5) Includes procedures to ensure that there are ~~practitioners~~ **licensed providers** and facilitators present at all times during an administration session with a minimum of one ~~practitioner~~ **licensed provider** and one facilitator for individual patient sessions; and in group administration sessions a minimum of one ~~practitioner~~ **licensed provider** for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated.

SOURCE Permit title only. Metz recommendation, page 1, recommendation 1. Addendum C. No ratio is amended.

Please review. No ratio is amended here, and no training permit is proposed anywhere in this draft. At the July 17, 2026 Training and Education Committee meeting the department stated that a training permit “really wouldn’t be necessary with the model that we have” because a student would already be covered “as them being a registered student with an educational program” and undertook to look at counting students toward the ratio “rather than needing to create an entire other permit or certification level.” This paragraph is the result. The phrase “training permit” appears nowhere in the rule as published; the qualification parameter is the “qualified student” definition in this paragraph.

7.35.3.29 Proposed new section. Practicum participants who are not qualified patients

Proposed new section. The first stage of the practicum, conducted with participants who are not qualified patients.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, practicum participants who are not qualified patients

There is no current language. This provision does not exist in the proposed rule as published.

Entire section, practicum participants who are not qualified patients

7.35.3.29 PRACTICUM PARTICIPANTS WHO ARE NOT QUALIFIED PATIENTS:
A. Purpose: This section provides for the stage of the practicum described in Paragraph (1) of Subsection D of 7.35.3.19 NMAC to be conducted with practicum participants who are not qualified patients, so that a student gains supervised exposure to non-ordinary states before working with clinical populations.
B. Practicum participant: A practicum participant is an individual who is not a qualified patient and who takes part in a practicum session in a retreat or peer-support model. Preparation and integration sessions are required for each participant.
C. Effect: This section applies only to the extent that the Medical Psilocybin Act, Sections 26-2D-1 through -11 NMSA 1978, authorizes the administration of medical psilocybin to a person who is not a qualified patient.

SOURCE Metz recommendation, page 3, step 1: "Initial facilitation experience with well participants (approximately 30 hours). Two to three sessions as a facilitator in a retreat or peer-support model."

Please review. Section 5(B)(2) of the Medical Psilocybin Act reaches "a clinician administering or a qualified patient taking psilocybin in an approved setting". Section 3(H) defines a qualified patient by diagnosis with a qualifying condition. A participant who is not a qualified patient falls outside both. Subsection C conditions this section accordingly.

Addendum A Provisions on which the practicum operates

Provisions on which 7.35.3.19 operates, and whether this draft amends each.

Provision	Page	What the practicum needs from it	In this draft
7.35.2.7 NMAC	n/a	Supplies every defined term used in Part 3, by force of 7.35.3.7	Not amended here. The permit retile would require amending the definition at P(7); 7.35.2 NMAC is a separate rulemaking. Facilitator, healing center, certifying clinician and student are used in Part 3 and defined in neither part. Addenda B and C
Medical Psilocybin Act, Section 5	n/a	The criminal and civil exemption. Names producers, clinicians and qualified patients, and protects presence only for anyone else	Statute. Not reachable by rule
7.35.3.9 C(3), E, F	3	Certification applications require documentation of the completed practicum, and E(1) sets the practitioner licensure predicate	Not amended
7.35.3.10 D	5	Out-of-jurisdiction 40-hour practicum waiver	Not amended. Disagrees with 7.35.3.19 K
7.35.3.11 A, B	5 to 7	Healing centers and other approved locations. The practicum may only happen in them	Not amended
7.35.3.12 A	7	Educational program certification	Not amended
7.35.3.13 B	8	Facilitator scope of work, which limits what the practicum can usefully train	Not amended
7.35.3.14	9	Authority to possess and administer	AMENDED. Heading, Subsections A and B
7.35.3.15 A(4)	9	Practicum site agreements need prior written approval, with no deadline on the department	Not amended
7.35.3.17 A	10 to 11	The 10 mentoring hours the consultation requirement would replace	Not amended
7.35.3.17 B, C	11	Test-out, graduation conditions, and the practicum records a program must keep	Not amended
7.35.3.18	11 to 12	The didactic requirement the practicum entry gate is measured against	AMENDED. Heading and Subsections A, C, E, F and G
7.35.3.19	12 to 13	The practicum	AMENDED. Re-lettered A to K
7.35.3.20 D	14	Students may be present at an administration session	Not amended
7.35.3.20 H(5)	14	Staffing ratio, which counts a qualified student	AMENDED. Permit title only. No ratio changed
7.35.3.22	15	Prohibitions, reaching "the qualified patient or certificant"	Not amended

7.35.3.27	16 to 19	Discipline and appeal	Not amended
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Proposed new provisions

Provision	What it does	Source	If dropped
7.35.3.18 H	States the 84-hour total in one place	Metz recommendation, page 3	Module hours survive individually; no total is stated anywhere
7.35.3.19 D	The four-step practicum sequence	Metz recommendation, pages 3 to 4; Wilson redline for the drafting	Practicum totals survive; the sequence is lost
7.35.3.19 F	Separates medical screening from the training program overseeing the practicum	Wilson redline	A program may clear its own students for the treatments that generate their hours
7.35.3.19 H	20 consultation hours, two presented cases, standardized evaluation form	Metz recommendation, pages 4 to 5	7.35.3.17 A survives at 10 hours with no requirement to have seen a client
7.35.3.19 I	End-of-life checkpoint	Metz recommendation, page 5	No end-of-life competency gate remains
7.35.3.29	Practicum participants who are not qualified patients	Metz recommendation, step 1, page 3	The first stage of the practicum has no lawful setting

No training permit is proposed. At the July 17, 2026 Training and Education Committee meeting the department stated that a training permit "really wouldn't be necessary with the model that we have" and undertook to count students toward the staffing ratio instead. Paragraph (5) of Subsection H of 7.35.3.20 as published does that. See the note at that paragraph.

Addendum B Terms used in Part 3 and defined nowhere

7.35.3.7 provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." 7.35.2 NMAC was adopted effective June 23, 2026. The terms below are used in Part 3. Apart from the permit title, no amendment to the definitions is proposed in this document.

Term used in Part 3	In 7.35.2.7?	What 7.35.2.7 has	Consequence
Facilitator	No. Zero occurrences in 7.35.2 NMAC	"Guide" an individual who has completed training and education approved by the department to be able to assist practitioners during the administration sessions and who has been registered with the department	Every facilitator provision in Part 3 rests on an undefined term. A guide holds no professional license, so a facilitator is not a "clinician" under Section 3(B) of the Act, while 7.35.3.14 B authorizes facilitators to possess and provide
Healing center	No	"Approved location" means a location approved by the department for psilocybin administration sessions	Healing centers are certified and regulated throughout Part 3 with no definition
Certifying clinician	No	"Clinician" means an approved health care provider licensed in New Mexico who holds a certification from the department to provide medical services to qualified patients	Probably the same role renamed
Student	No	Nothing	Carries consequence at 7.35.3.19, 7.35.3.20 D and 7.35.3.20 H(5)
Practitioner	Yes	"Practitioner" means an individual who is a licensed healthcare professional who is certified by the department to provide medical psilocybin integrative therapy, supervise guides, and who has completed department required trainings	Carries the same licensure predicate as the Act's "clinician". Retitled by this draft. Addendum C

Addendum C The permit title, and every place a retile reaches

Recommendation 1 of the Metz recommendation, page 1, proposes retitling "Practitioner" as "Licensed Provider". This draft carries the retile. It is marked ~~practitioner~~ licensed provider wherever it falls inside a provision reproduced in this document. The term is defined at Paragraph (7) of Subsection P of 7.35.2.7 NMAC, and 7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." A retile is therefore an amendment to 7.35.2 NMAC plus a conforming pass over 7.35.3 NMAC. The tables below count every occurrence in both parts, so that the committee can see the whole surface.

Two drafting notes. First, the Metz recommendation writes "Licensed Provider" in title case; this draft writes "licensed provider" in lower case, because the rule as published writes "practitioner" in lower case except in headings and at the start of a subsection. Second, the retile reaches no hour count and no practicum requirement. Those attach to the permit type, and what the permit is turns on the licensure predicate at Paragraph (1) of Subsection E of 7.35.3.9 NMAC, which already requires a New Mexico professional license. Adopting or declining the retile changes nothing else in this document.

7.35.3 NMAC, section	Occurrences	Reached by this draft	Note
Part title	1	No	"PART 3 PATIENTS, CERTIFYING CLINICIANS, PRACTITIONERS, FACILITATORS, HEALING CENTERS, OTHER APPROVED LOCATIONS, AND EDUCATIONAL PROGRAMS"
7.35.3.2 Scope	1	No	Who the part applies to
7.35.3.6 Objective	1	No	
7.35.3.9 Certification requirements	5	No	Includes the section heading and the licensure predicate at E(1)
7.35.3.10 Application process	5	No	Includes the section heading and the out-of-jurisdiction pathway
7.35.3.11 Healing centers	3	No	
7.35.3.12 Educational programs	1	No	
7.35.3.13 Requirements and prohibitions	12	No	The largest block outside this draft. Includes the section heading and the scope of work
7.35.3.14 Authorized possession	6	Yes, all 6	Heading and Subsection A
7.35.3.17 Educational programs	2	No	Includes the mentoring provision at Subsection A
7.35.3.18 Educational requirements	14	Yes, all 14	Heading and Subsections A, C, E, F and G
7.35.3.19 Practicum requirements	6	Yes, all 6	Heading and Subsections A and C
7.35.3.20 Healing centers	3	Yes, all 3	All three are in Paragraph (5) of Subsection H
7.35.3.26 Voluntary withdrawal	1	No	

7.35.3.27 Disciplinary actions	5	No	Includes the section heading
Total, 7.35.3 NMAC	66	29 reached, 37 not	Fourteen of the twenty-eight sections carry the term

7.35.2 NMAC, section	Occurrences	Reached by this draft	Note
7.35.2.7 Definitions	3	No	The definition itself at P(7); also inside the definitions of "certification" at C(3) and "guide" at G(3)
7.35.2.10 General producer requirements	1	No	
7.35.2.14 Packaging and labeling	3	No	The product information document duties
7.35.2.15 General tracking requirements	1	No	
7.35.2.19 Required testing	2	No	Includes the failed-lot notification duty
7.35.2.24 Transportation	1	No	
7.35.2.26 Disciplinary actions	1	No	
Total, 7.35.2 NMAC	12	0 reached	7.35.2 NMAC was adopted effective June 23, 2026 and is not before this hearing

Please review. The retitle is all or nothing. If it is adopted, the conforming amendment has to reach all 66 occurrences in 7.35.3 NMAC and all 12 in 7.35.2 NMAC, including the definition at Paragraph (7) of Subsection P of 7.35.2.7 NMAC. This document reaches 29. If the committee adopts the retitle, the remaining 49 are a conforming pass, and 7.35.2 NMAC is a separate rulemaking. If the committee declines it, strike every ~~practitioner~~ licensed provider mark in this document; nothing else in the document depends on it.

Addendum D Sections not addressed

Part 3 has twenty-eight sections. This draft addresses the practicum and the provisions on which it operates. The sections below are not addressed. Findings are recorded in the concerns inventory of July 23, 2026, which lists five blocking and twenty-three material findings across the part.

Group	Sections	Recorded findings
Terminology	7.35.3.7, and 7.35.2.7 NMAC	Facilitator, healing center, certifying clinician and student undefined. Addendum B
Educational programs	7.35.3.12, .15, .16, .17	The evaluator conflict rule disqualifies the evaluators a program must hire; the third-party evaluation deferral expires the day it becomes available; no deadline binds the department on instructor changes; the denial grounds were deleted; the evaluator qualification standard may be unmeetable; mentoring is unfunded and untied to certification; the test-out price cap assumes per-module pricing
Out-of-jurisdiction pathway	7.35.3.10	The two 40-hour waivers disagree; the 2027 waiver may cancel itself; the approved list is empty at launch and populates only retroactively; individuals must produce an evaluation only a program can commission
Locations and oversight	7.35.3.11, .20, .21	Healing center staff authority hangs on a registration that does not exist; outdoor locations need no AED while healing centers do; the patient roster and the patient-interview authority; other approved locations have a 90-day term and no renewal path
Patients, applicants, process	7.35.3.8, .9, .13, .22 to .27	Facilitator scope conflicts with the practicum and the ratios; complainants lose confidentiality; the benefit-risk attestation; renewals can lapse while pending; the re-application clause reads as a deadline; two review tracks with inconsistent scope; a suspended certificant can be out of practice more than 135 days
Mechanical	Throughout	Five section headings read 7.34.3 instead of 7.35.3, two of which are corrected here; two sections carry a real effective date while the rest carry placeholders; a numbering gap at 7.35.3.9 D

Sources. Rule as published: *rules-draft-2026-07-23-published.pdf*, 19 pages. Recommendations: Dr. Anne Metz, *Recommendations on Education and Training Requirements for Facilitators and Licensed Providers*, July 17, 2026, with the one-page summary and the committee slide deck of the same date. Working redline: Denali Wilson, *NMAC 7.35.3*, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION." Medical Psilocybin Act, Senate Bill 219, 2025, as introduced; the enacted text at Sections 26-2D-1 through -11 NMSA 1978 has not been checked against it. 7.35.2 NMAC as adopted effective June 23, 2026.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and no other alteration. Each block was checked against the text layer of the published rule.

Status. Working draft v7. Not a filing, not submitted, and not adopted rule text.